

Patient 10**Report A**

CT BRAIN

CLINICAL NOTES:

Acute onset of confusion associated with dysarthria. Patient also had a fall this afternoon with abnormal GCS.

TECHNIQUE: Axial CT images are been acquired at 1 mm collimation without intravenous contrast medium. They are reviewed in multiple planes.

FINDINGS:

There are generalised changes of moderate cerebral atrophy, There is no sign of intracranial bleeding or mass, Midline structures are not displaced. The basal cisterns are preserved. The cerebellum has normal configuration,

The bones on bone windows are clear.

A subcutaneous haematoma is demonstrated over the left parieto-occipital region.

CONCLUSION:

No sign of intracranial bleeding.

Report B

6514017,31105/2016 MR Head

6514017 31/05/2016 MR MRA

6514017,31105/2016 MR Carotids

Clinical indications: 1 day history of confusion. Possible stroke or encephalitis.

Report:

Comparison is made with the CT dated 30/05/2016. There are patchy areas of diffusion restriction in keeping with acute MCA territory infarcts involving the left inferior parietal and temporal lobe. Several additional foci of T2/FLAIR hyperintensity are demonstrated in the cerebral white matter and in the pons, in keeping with a moderate degree of small vessel ischaemia. There are tiny mature infarcts in the left cerebellar hemisphere. A small focus of susceptibility artefact in the left frontal corona radiata most likely represents a microhaemorrhage.

Flow within the main intracranial arteries appears preserved. Note is made of a slightly bulbous appearance to the anterior Communicating artery complex, but a discrete aneurysm is not clearly delineated. On the three-dimensional reformats, the differential for the appearances may include a fenestration. No significant stenosis is demonstrated within the scanned extracranial arteries.